

CHRONIC ARTHRITIS

- If patient has discrete episodes of joint pain and swelling that completely resolve in between, consider gout →152.
- The patient with chronic arthritis has had continuous joint pain for at least 6 weeks. Distinguish mechanical osteoarthritis from inflammatory rheumatoid arthritis as follows:

Osteoarthritis likely if:

- Affects joints only.
- Weight-bearing joints and possibly hands and feet
- Joints may be swollen but not warm.
- Stiffness on waking lasts less than 30 minutes.
- Pain is worse with activity and gets better with rest.

Rheumatoid (inflammatory) arthritis likely if:

- May be systemic: weight loss, fatigue, poor appetite, muscle wasting
- Hands and feet are mainly involved.
- Joints are swollen and warm.
- Stiffness on waking lasts more than 30 minutes.
- Pain and stiffness get better with activity.

If rheumatoid arthritis likely or uncertain of diagnosis, refer for specialist assessment.

Assess the patient with chronic arthritis

Assess	When to assess	Note
Symptoms	Every visit	Manage symptoms as on symptom pages. Assess and advise on chronic pain ↗ 61. If difficulty sleeping ↗ 87.
Activities of daily living	Every visit	Ask if patient can walk as well as before, can cope with buttons and use knife and fork properly.
Depression	Every visit	In the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either ↗ 143.
Joints	Every visit	Look for warmth, tenderness and limitation in range of movement of joints.
BMI	At diagnosis	BMI = weight (kg) ÷ height (m) ÷ height (m). BMI > 25 puts stress on weight-bearing joints. If BMI > 25, assess CVD risk ↗ 127.
HIV	At diagnosis	Test for HIV ↗ 110.

Advise the patient with chronic arthritis

- If BMI > 25, advise to reduce weight to decrease stress on weight-bearing joints like knees and feet.
- Encourage the patient to be as active as possible, but to rest with acute flare-ups. If patient smokes, encourage to stop ↗ 141. Refer to support group/helpline ↗ 178.
- Ensure the patient using disease modifying medication knows to have regular blood monitoring depending on the prescribed medications from the specialist clinic.

Health for All

↗ 128

Treat the patient with chronic arthritis

- Give **methyl salicylate ointment** to apply to affected joints.
- If **osteoarthritis**: give **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) as needed. If better, reduce dose to 500mg 6-8 hourly as needed.
 - If no response to paracetamol and inflammation present, give **ibuprofen**¹ 400mg 8 hourly with food as needed for 7 days. If > 65 years, previous peptic ulcer, on aspirin, warfarin or prednisone, also give **lansoprazole** 30mg daily for 7 days.
 - Refer to doctor if available to consider steroid injection/s.
- If **rheumatoid (inflammatory) arthritis**: refer to specialist to confirm diagnosis. Rheumatoid arthritis must be treated early with disease modifying anti-rheumatic medication to control symptoms, preserve function, and minimise further damage.
 - While awaiting appointment, give **ibuprofen**¹ 400mg 8 hourly with food for up to 3 months. If > 65 years, previous peptic ulcer, on aspirin or prednisone, also give **lansoprazole** 30mg daily to take while on ibuprofen.
 - If confirmed diagnosis and **acute flare** (symptoms much worse): refer. While waiting for appointment, give **ibuprofen**² 400mg 8 hourly with food for up to 2 weeks. Avoid ibuprofen if peptic ulcer, asthma, hypertension, heart failure, kidney disease or on warfarin: give instead **prednisone** 7.5mg daily for up to 2 weeks.
- If rheumatoid arthritis, or if osteoarthritis with difficulty with activities of daily living, refer to physiotherapist or occupational therapist.

Review monthly until symptoms controlled, then 3-6 monthly. If poor response to treatment, refer to specialist.

¹Avoid if peptic ulcer, asthma, hypertension, heart failure or kidney disease or on warfarin, discuss instead. If patient also taking aspirin, advise to wait at least 30 minutes after taking aspirin before taking ibuprofen. ²If > 65 years, previous peptic ulcer, on aspirin or prednisone, also give **lansoprazole** 30mg daily to take while on ibuprofen.